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NCCN Guidelines Version 1.2027
Small Cell Lung Cancer

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Discussion

Updates in Version 1.2027 of the NCCN Guidelines for Small Cell Lung Cancer:

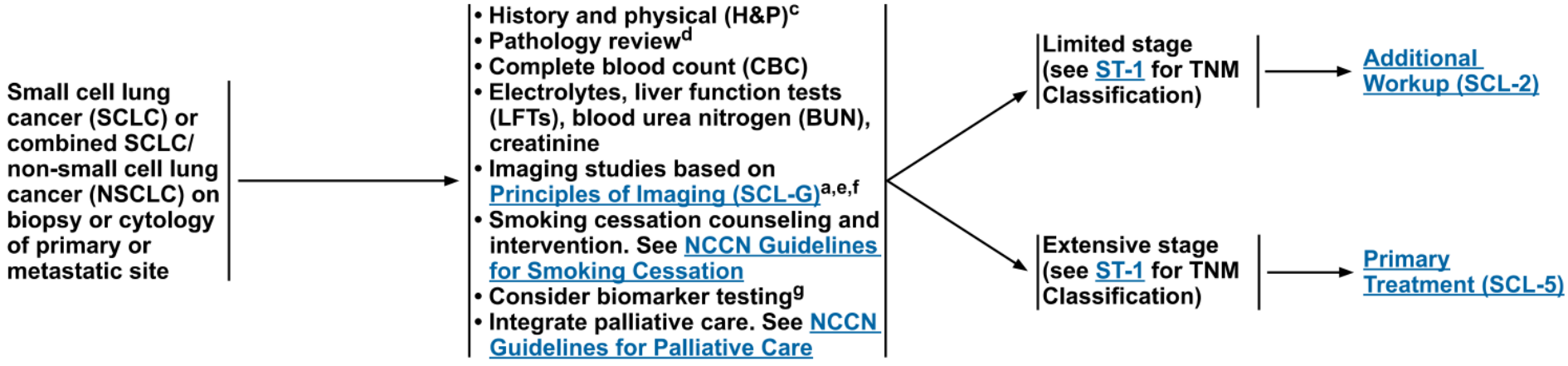
SCL-E 3 of 6

- Footnote d: Contraindications for treatment with PD-1/PD-L1 inhibitors may include active or previously documented autoimmune disease and/or concurrent use of immunosuppressive agents. If TKI is continued, *for transformed SCLC from NSCLC with oncogenic driver,* ICI should be avoided due to known toxicity.
- Footnote added: NCCN Guidelines for Management of CAR T-Cell and Lymphocyte Engager-Related Toxicities.
- Footnote added: Pembrolizumab and berahyaluronidase alfa-pmph subcutaneous injection may be substituted for IV pembrolizumab. Pembrolizumab and berahyaluronidase alfa-pmph has different dosing and administration instructions compared to IV pembrolizumab.

MS-1

- Discussion section updated to reflect changes within the algorithm

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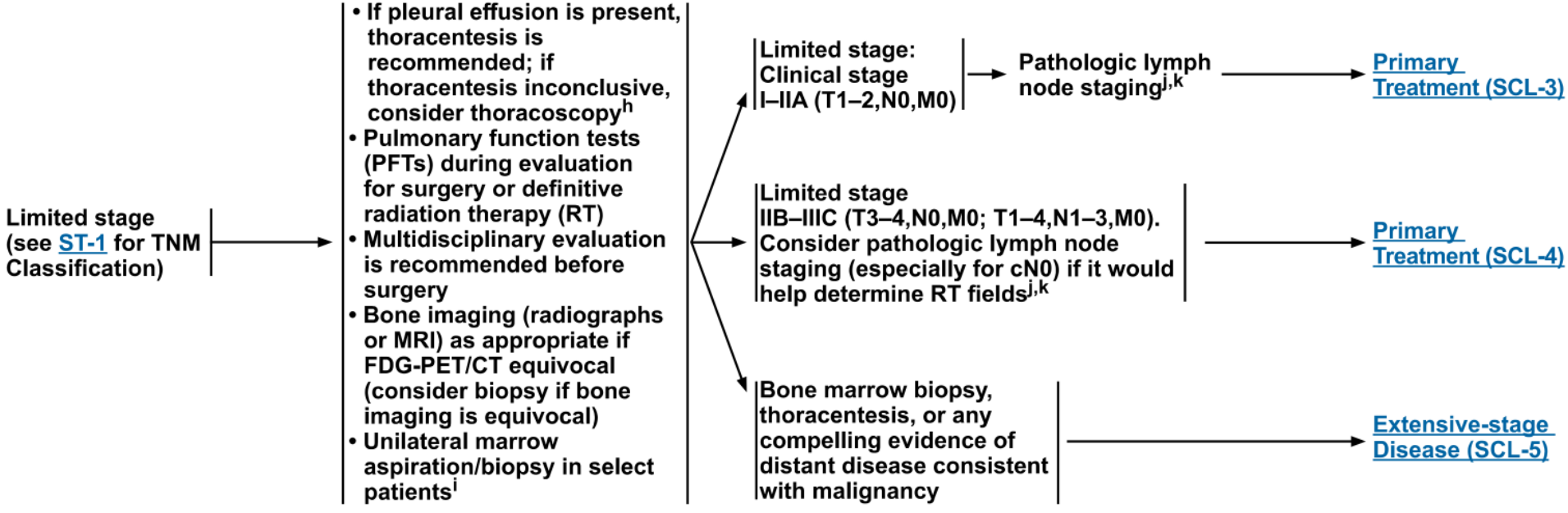
[a] If extensive stage is established, further staging evaluation is optional and dependent on the clinical situation. However, brain imaging—MRI (preferred) or CT with contrast—is recommended.
[b] Workup of SCLC should be expedited, with studies done in parallel whenever possible.
[c] Signs and Symptoms of Small Cell Lung Cancer (SCL-A).
[d] Principles of Pathologic Review (SCL-B).
[e] Brain MRI is more sensitive than CT for identifying brain metastases and is preferred over CT. See Principles of Imaging (SCL-G).
[f] If FDG-PET/CT is not available, bone scan may be used to identify bone metastases. Pathologic confirmation is recommended for isolated or equivocal lesions if their involvement would change clinical management.
[g] Biomarker testing via blood, tissue, or both can be considered in rare cases—particularly for patients with extensive-stage/relapsed SCLC who do not smoke tobacco, lightly smoke, have remote smoking history, or have diagnostic or therapeutic dilemma, or at time of relapse—if not previously done, because this may change management.

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STAGE | **ADDITIONAL WORKUP[b]**

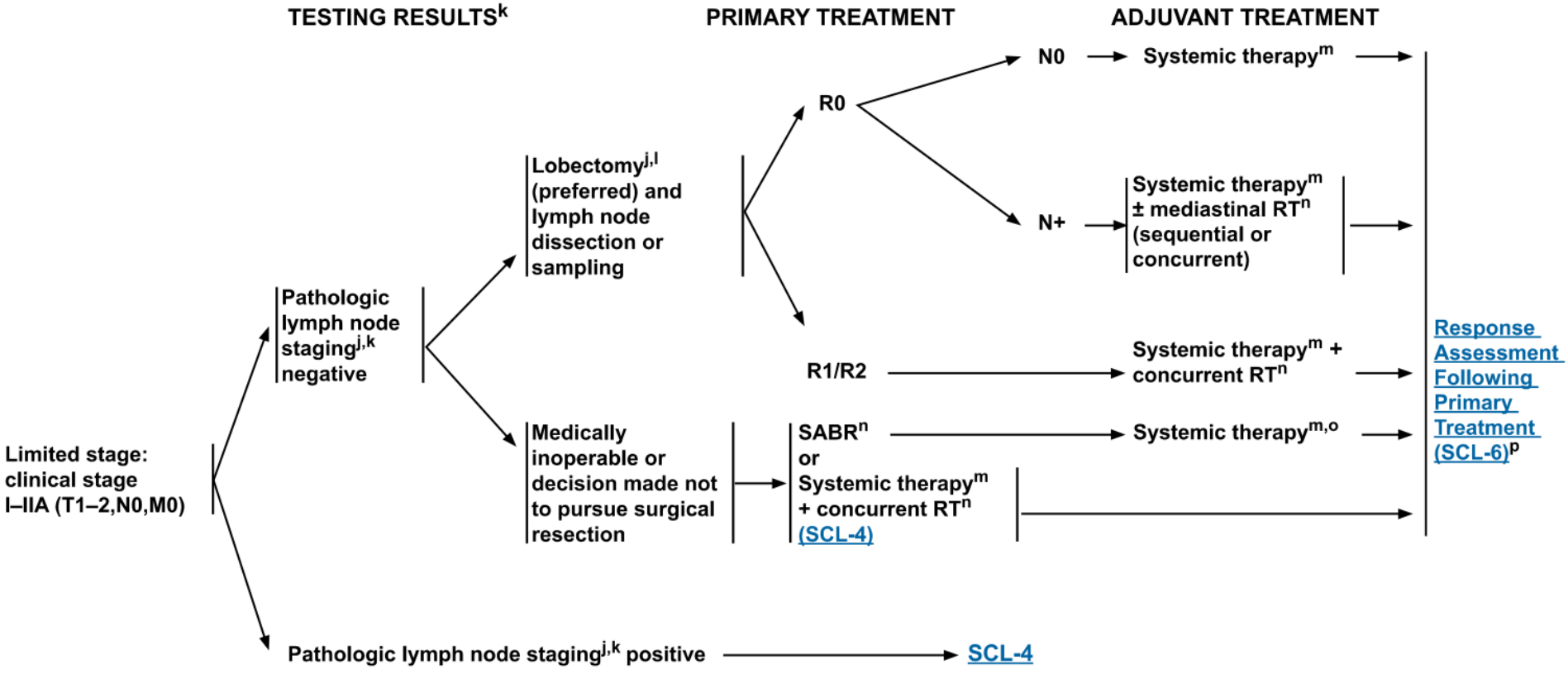


[b] Workup of SCLC should be expedited, with studies done in parallel whenever possible.
[h] While most pleural effusions in patients with lung cancer are due to tumor, there are a few patients in whom multiple cytopathologic examinations of pleural fluid are negative for tumor and fluid is non-bloody and not an exudate. When these elements and clinical judgment dictate that the effusion is not related to the tumor, the effusion should be excluded as a staging element. Pericardial effusion is classified using the same criteria.
[i] Selection criteria include: nucleated red blood cells (RBCs) on peripheral blood smear, neutropenia, or thrombocytopenia suggestive of bone marrow infiltration. Perform bone marrow biopsy only if it changes clinical management.
[j] Principles of Surgical Resection (SCL-C).
[k] Lymph node staging procedures include endobronchial ultrasound-guided biopsy (preferred, if expertise exists), mediastinoscopy, mediastinotomy, esophageal ultrasound-guided biopsy, and video-assisted thoracoscopy. If nodal involvement is established by one modality of mediastinal staging, additional modalities are not necessary, but staging biopsies of visible nodes in all accessible nodal stations is encouraged to inform radiotherapy targeting.

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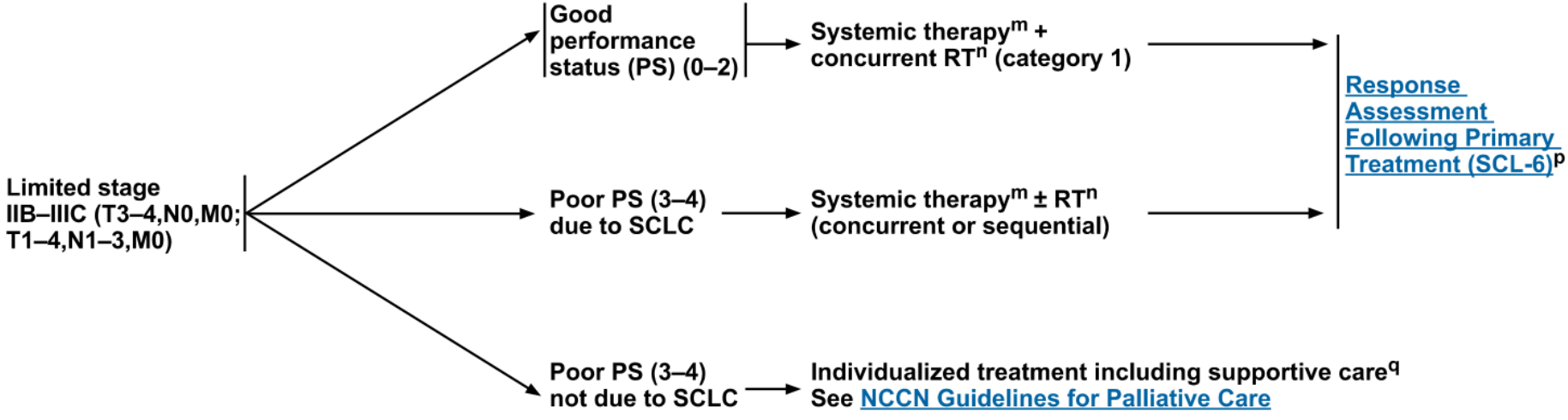
[j] Principles of Surgical Resection (SCL-C).
[k] Lymph node staging procedures include endobronchial ultrasound-guided biopsy (preferred, if expertise exists), mediastinoscopy, mediastinotomy, esophageal ultrasound-guided biopsy, and video-assisted thoracoscopy. If nodal involvement is established by one modality of mediastinal staging, additional modalities are not necessary, but staging biopsies of visible nodes in all accessible nodal stations is encouraged to inform radiotherapy targeting.
[l] Select patients may be treated with systemic therapy/RT as an alternative to surgical resection.
[m] Principles of Systemic Therapy (SCL-E).
[n] Principles of Radiation Therapy (SCL-F).
[o] Systemic therapy may be initiated first if time to initiation of stereotactic body radiotherapy (SABR) will be prolonged.
[p] Principles of Imaging (SCL-G).

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PRIMARY TREATMENT

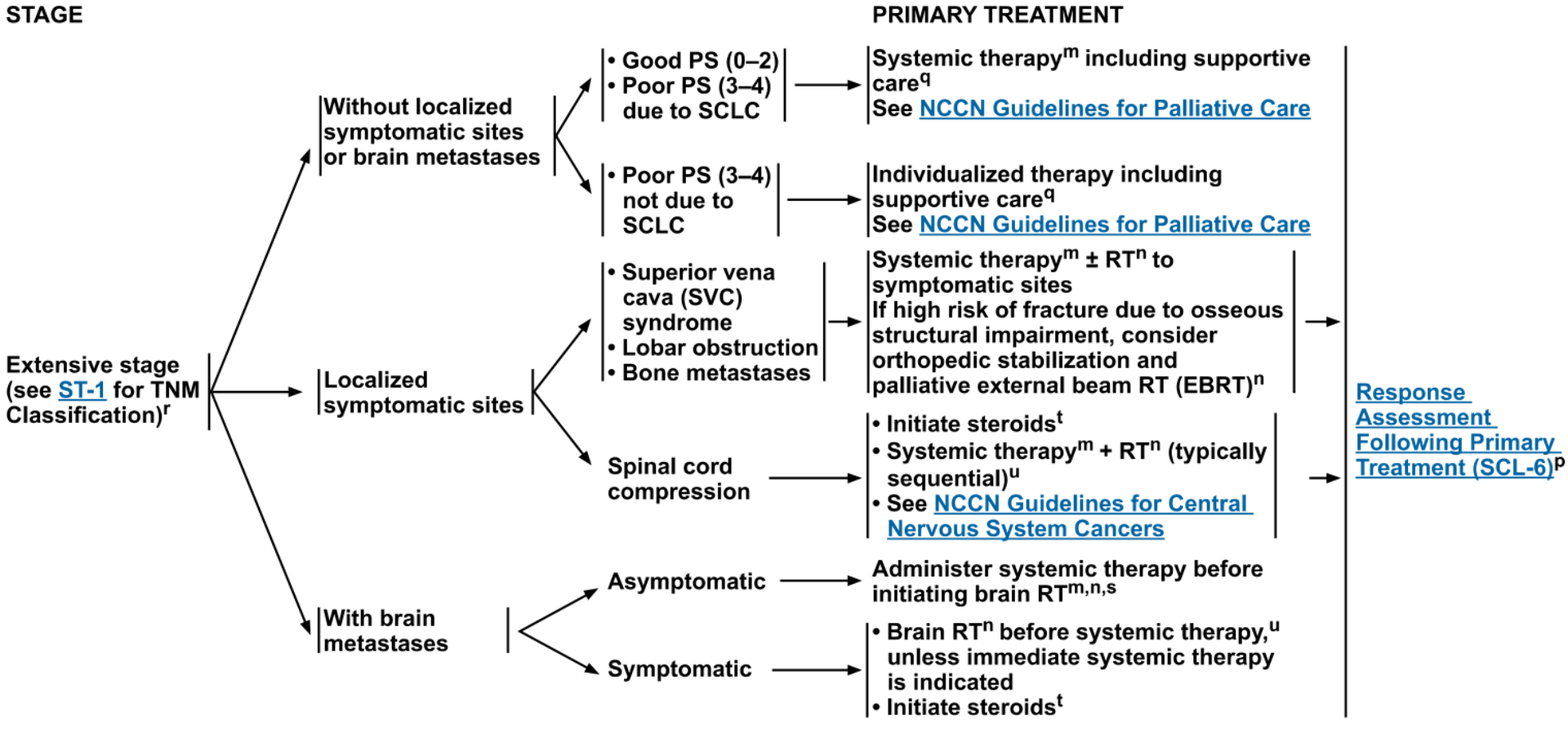


[m] Principles of Systemic Therapy (SCL-E).
[n] Principles of Radiation Therapy (SCL-F).
[p] Principles of Imaging (SCL-G).
[q] Principles of Supportive Care (SCL-D).

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m Principles of Systemic Therapy (SCL-E).
n Principles of Radiation Therapy (SCL-F).
p Principles of Imaging (SCL-G).
q Principles of Supportive Care (SCL-D).
r For transformation to SCLC from NSCLC, consider referral to a center with expertise (SCL-E 4 of 6).
s If brain metastases progress while on systemic therapy, it is recommended that brain RT is initiated before completion of systemic therapy.
t Initiate steroids for patients with symptomatic neurologic disease. Eg, dexamethasone 10 mg loading dose followed by 4–6 mg maintenance dose (IV or PO every 4–6 hours [or as appropriate]). Kumar A, et al. Clin Spine Surg 2017;4:156-163.
u With neurologic symptoms, RT is preferred before systemic therapy. Systemic therapy may start first if RT cannot be started expeditiously or if controlling systemic symptoms is more urgent.

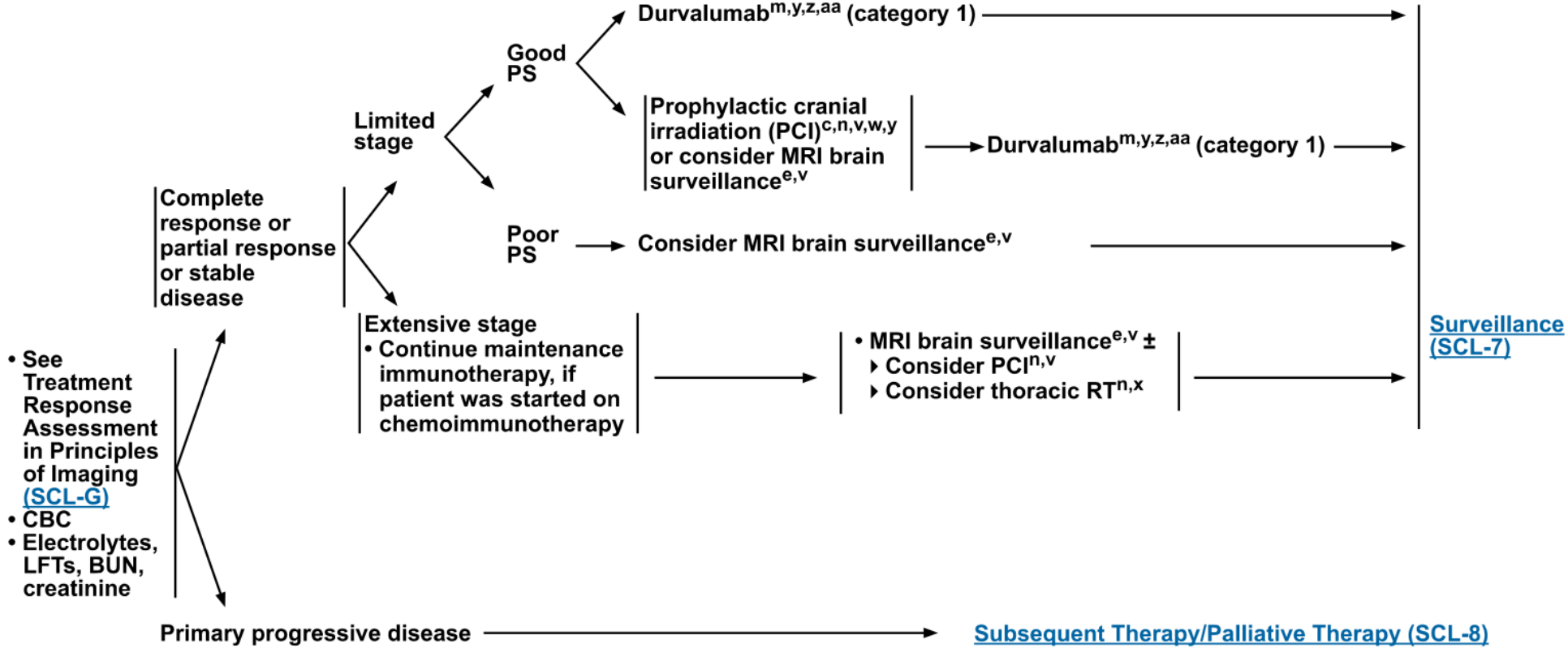
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RESPONSE ASSESSMENT FOLLOWING PRIMARY TREATMENT

ADJUVANT THERAPY



Footnotes

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FOOTNOTES

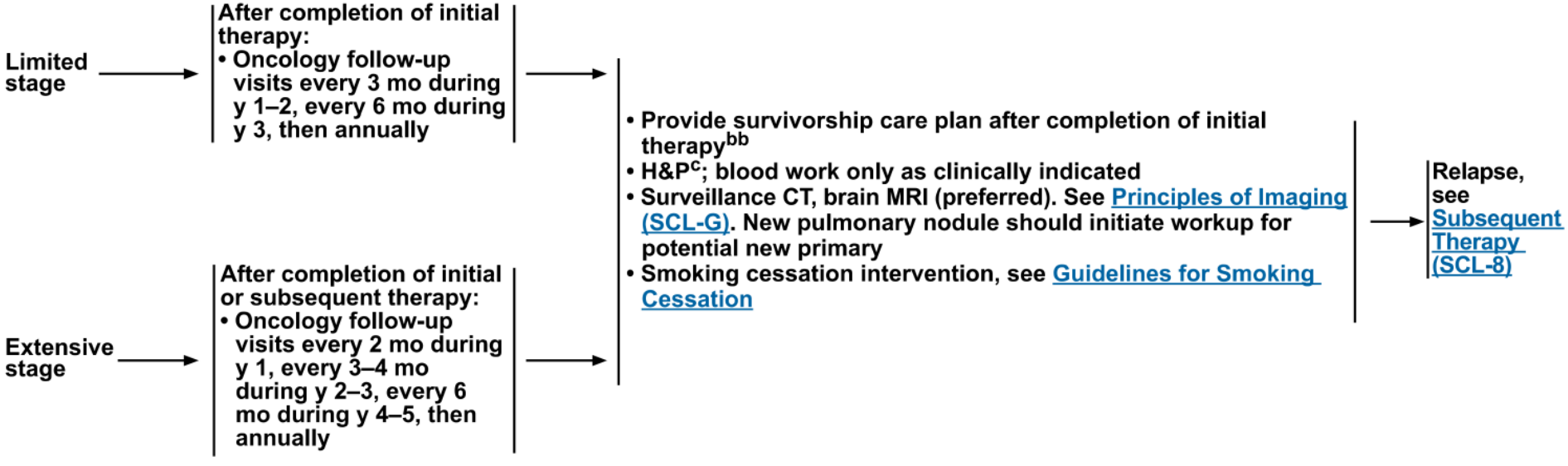
[c] Signs and Symptoms of Small Cell Lung Cancer (SCL-A).
[e] Brain MRI is more sensitive than CT for identifying brain metastases and is preferred over CT. See Principles of Imaging (SCL-G)
[m] Principles of Systemic Therapy (SCL-E).
[n] Principles of Radiation Therapy (SCL-F).
[v] PCI is not recommended in patients with poor PS or impaired neurocognitive function. Increased cognitive decline after PCI has been observed in older adults (≥60 years) in prospective trials; the risks and benefits of PCI versus close brain surveillance, MRI (preferred), or CT with contrast should be carefully discussed with these patients. Edelman MJ, Am Soc Clin Oncol Educ Book 2020:40:24-28.
[w] The benefit of PCI is unclear in patients who have undergone definitive therapy for pathologic stage I (T1–2a,N0,M0) SCLC. See Principles of Radiation Therapy (SCL-F).
[x] Sequential RT to thorax in selected patients, especially with residual thoracic disease and low-bulk extrathoracic metastatic disease that has responded to systemic therapy.
[y] If PCI is considered, it should be given prior to durvalumab.
[z] For those with good PS who are medically inoperable or decision was made not to pursue surgical resection.
[aa] Imaging every 8 weeks for the first 18 months and every 12 weeks after.

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SURVEILLANCE



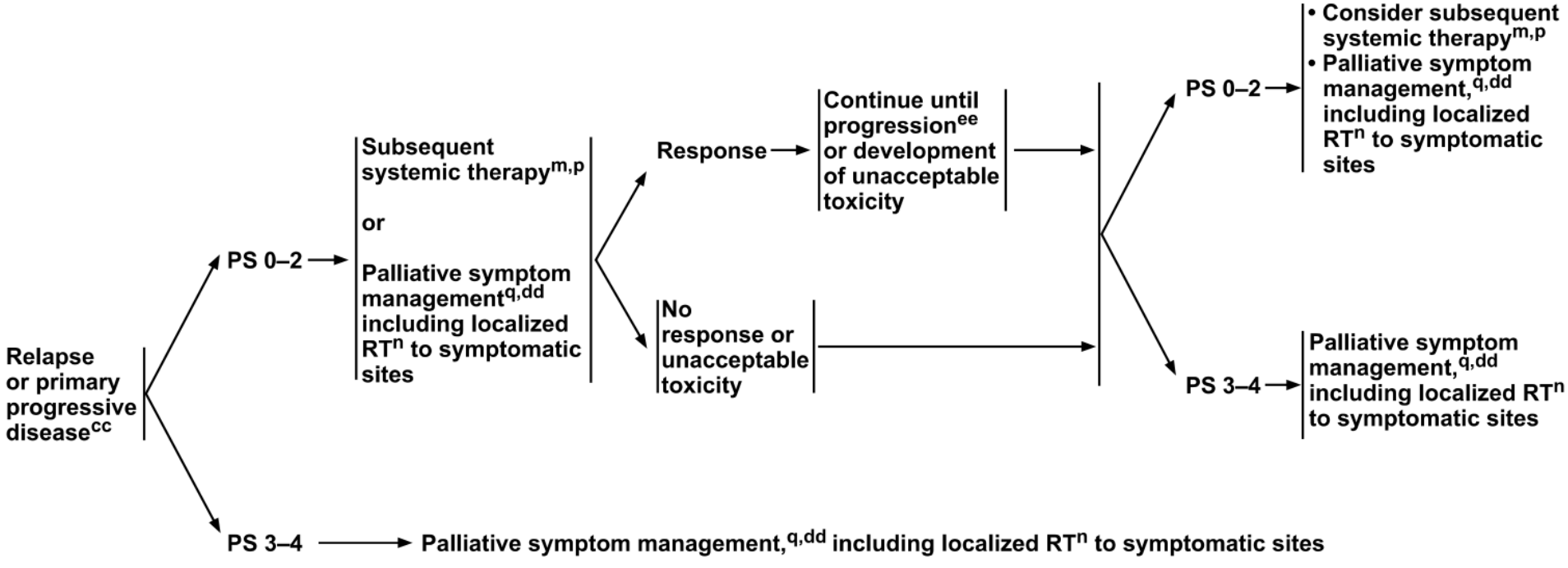
[c] Signs and Symptoms of Small Cell Lung Cancer (SCL-A).
[bb] NCCN Guidelines for Survivorship.

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PROGRESSIVE DISEASE **SUBSEQUENT THERAPY/PALLIATIVE THERAPY**



[m] Principles of Systemic Therapy (SCL-E).
[n] Principles of Radiation Therapy (SCL-F).
[p] Principles of Imaging (SCL-G).
[q] Principles of Supportive Care (SCL-D).
[cc] Consider biomarker testing, if not previously done, to determine clinical trial eligibility.
[dd] NCCN Guidelines for Palliative Care.
[ee] For central nervous system (CNS) progression only, continue systemic therapy and treat the brain metastases with RT. See Principles of Radiation Therapy (SCL-F).

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SIGNS AND SYMPTOMS OF SMALL CELL LUNG CANCER

Signs and Symptoms Due to Local Primary Tumor Growth
- Cough – endobronchial irritation, bronchial compression
- Hemoptysis – usually central or cavitary lesion
- Wheezing – partially obstructing endobronchial lesion
- Fever – postoperative pneumonia
- Dyspnea – bronchial obstruction, pneumonia, pleural effusion

Signs and Symptoms Due to Primary Tumor Invasion or Regional Lymphatic Metastases
- Hoarseness – left vocal cord paralysis due to tumor invasion or lymphadenopathy in the aortopulmonary window
- Hemidiaphragm elevation – due to phrenic nerve compression
- Dysphagia – due to esophageal compression
- Chest pain – involvement of pleura or chest wall, often dull and non-localized
- SVC syndrome – due to local invasion into mediastinum or lymphadenopathy in right paratracheal region
- Pericardial effusion and tamponade
- Cervical or supraclavicular lymph node enlargement

Signs and Symptoms Due to Extrathoracic (Hematogenous) Metastases
- Brain metastases:
  - Headache, focal weakness or numbness, confusion, slurred speech, gait instability, incoordination
- Leptomeningeal carcinomatosis:
  - Headache, confusion, cranial nerve palsy, diplopia, slurred speech, radicular back pain, spinal cord compression
- Adrenal metastases:
  - Mid-back or flank pain, costovertebral angle tenderness
  - Adrenal insufficiency due to tumor involvement (rare)
- Liver metastases:
  - Right upper quadrant pain or tenderness, jaundice, fatigue, fever, hepatomegaly
- Bone metastases:
  - Bone pain
  - Spinal cord compression – back pain, muscle weakness, numbness, paresthesia, loss of bowel and bladder control
- Constitutional:
  - Anorexia/cachexia – weight loss
  - Fatigue

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Continued

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SIGNS AND SYMPTOMS OF SMALL CELL LUNG CANCER

Signs and Symptoms of Paraneoplastic Syndromes
- Presence does not imply metastases or incurability

Endocrine:
- Due to ectopic peptide hormone production
- Usually reversible with successful anti-tumor therapy
- Syndrome of inappropriate antidiuretic hormone secretion (SIADH)[a]:
  - Ectopic vasopressin (antidiuretic hormone, ADH) secretion
  - Clinically significant hyponatremia in 5%–10% of SCLC
  - Malaise, weakness, confusion, obtundation, volume depletion, nausea
  - Hyponatremia, euvolemia, low serum osmolality, inappropriately concentrated urine osmolality, normal thyroid and adrenal function
- Cushing syndrome[a]:
  - Ectopic adrenocorticotropic hormone (ACTH) secretion
  - Weight gain, moon facies, hypertension, hyperglycemia, generalized weakness
  - High serum cortisol and ACTH, hypernatremia, hypokalemia, alkalosis

Neurologic:
- All specific syndromes are rare
  - Subacute cerebellar degeneration – ataxia, dysarthria
  - Encephalomyelitis – confusion, obtundation, dementia
  - Sensory neuropathy – pain, sensory loss
  - Lambert-Eaton myasthenic syndrome (LEMS)[a] – proximal muscle weakness, autonomic dysfunction
    - ◊ Neurologic workup, preferably in consultation with neurology, may include but is not limited to tests for PQ- and N-type voltage-gated calcium channel (VGCC) antibodies.
  - Cancer-associated retinopathy – visual loss, photosensitivity
- Consider early subspecialty consultation for unusual paraneoplastic neurologic syndromes to ensure the most recent management is done
- If paraneoplastic neurologic syndrome is suspected, consider obtaining a neurologic consultation and/or comprehensive paraneoplastic antibody panel

Hematologic:
- Anemia
- Leukemoid reaction – leukocytosis
- Trousseau syndrome – migratory thrombophlebitis

[a] Principles of Supportive Care (SCL-D).

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PRINCIPLES OF PATHOLOGIC REVIEW

Pathologic Evaluation

- Pathologic evaluation is performed to determine the histologic classification of lung tumors and relevant staging parameters.
- The World Health Organization (WHO) tumor classification system provides the foundation for the classification of lung tumors, including histologic subtype, staging factors, clinical features, molecular characteristics, genetics, and epidemiology.[1-3]
- SCLC is a poorly differentiated neuroendocrine carcinoma. Distinguishing SCLC from other neuroendocrine tumors, particularly typical and atypical carcinoids, is important due to significant differences in epidemiology, genetics, treatment, and prognosis.[4-6]
- SCLC can be diagnosed on good-quality histologic samples via high-quality hematoxylin and eosin (H&E)-stained sections or on well-preserved cytologic samples.
  - SCLC is characterized by small blue cells with scant cytoplasm, high nuclear-to-cytoplasmic ratio, granular chromatin, and absent or inconspicuous nucleoli.
  - SCLC cells are round, oval, or spindle-shaped with molding and high mitotic counts.[7-9]
  - The most useful characteristics for distinguishing SCLC from large-cell neuroendocrine carcinoma (LCNEC) are the high nuclear-to-cytoplasmic ratio and paucity of nucleoli in SCLC.
- Careful counting of mitoses is essential, because it is the most important histologic criterion for distinguishing SCLC from typical and atypical carcinoids. Strongly recommend a second opinion—with a pathologist specializing in the diagnosis of thoracic malignancies—for diagnostic dilemma, including carcinoid.
  - SCLC (>10 mitoses/2 mm^2 field); atypical carcinoid (2–10 mitoses/2 mm^2 field); typical carcinoid (0–1 mitoses/2 mm^2 field)
  - Mitoses should be counted in the areas of highest activity and per 2 mm^2 field, rather than per 10 high-power fields.
  - In tumors that are near the defined cutoffs of 2 or 10 mitoses per 2 mm^2, at least three 2-mm^2 fields should be counted and the calculated mean (rather than the single highest mitotic count) should be used to determine the overall mitotic rate.[1,2]
- SCLC is often associated with necrosis. However, necrosis, usually punctate, is also seen in atypical carcinoid tumors. Counting mitotic figures helps to distinguish these two entities.
- Combined SCLC consists of both SCLC histology and NSCLC histology (squamous cell, adenocarcinoma, spindle/pleomorphic, and/or large cell). There is no minimal percentage of NSCLC histologic elements required; when any are present along with SCLC, this can be called combined SCLC, except in combination with LCNEC. At least 10% of the tumor should show LCNEC morphology to be classified as combined SCLC and LCNEC.[1]
- Biomarker testing via blood, tissue, or both can be considered in rare cases—particularly for patients with extensive-stage/relapsed SCLC who do not smoke tobacco, lightly smoke, have remote smoking history, or have diagnostic or therapeutic dilemma, or at time of relapse—if not previously done, because this may change management.

Immunohistochemical Staining

- Immunohistochemistry can be very helpful in diagnosing SCLC in limited samples.[5,7]
  - Nearly all SCLCs are positive for cytokeratin antibody mixtures with broad reactivity, such as AE1/AE3 and CAM5.2.[1,10]
  - The majority of SCLCs are reactive to markers of neuroendocrine differentiation, including insulinoma-associated protein 1 (INSM1), CD56/NCAM, synaptophysin, and chromogranin A. Fewer than 5% of SCLCs are negative for all neuroendocrine markers.[11,12] For cases with suspicious SCLC morphology without expression of neuroendocrine markers, POU2F3 immunohistochemical staining can be considered.[13,14]
  - Thyroid transcription factor-1 (TTF-1) is positive in 85% to 90% of SCLCs.[15-18]
  - Additional immunohistochemical markers are useful in distinguishing small cell carcinoma from poorly differentiated non-small cell carcinoma and combined carcinoma using Napsin A as a marker of adenocarcinoma, and p40 or p63 as a marker of squamous differentiation.[10] It should, however, be noted that p40 and p63 can be focally positive in small cell carcinoma.
- Ki-67 immunostaining can be very helpful in distinguishing SCLC from carcinoid tumors, especially in small biopsy samples with crushed or necrotic tumor cells in which counting mitotic figures is difficult.[4,5]
  - The Ki-67 proliferative index in SCLC is typically 50% to 100%.[1]
- Carcinoid tumors generally retain RB1 expression and show a wild-type p53 staining pattern. In contrast, most high-grade neuroendocrine carcinomas demonstrate loss of RB1 expression and/or aberrant (mutation-type) p53 expression, characterized by either diffuse strong nuclear staining (overexpression) or complete absence of staining (null pattern).

References

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PRINCIPLES OF SURGICAL RESECTION

- **Stage I–IIA SCLC is diagnosed in <5% of patients with SCLC.**
- **Patients most likely to benefit from surgery are those with SCLC that is clinical stage I–IIA (T1–2,N0,M0) after standard staging evaluation (including CT of the chest and upper abdomen, brain imaging, FDG-PET/CT imaging, and lymph node staging).[1,2]**
  - **Prior to resection, all patients should undergo mediastinoscopy or other surgical mediastinal staging to rule out occult nodal disease. This may also include an endoscopic staging procedure.**
  - **For patients undergoing definitive surgical resection, the preferred operation is lobectomy with mediastinal lymph node dissection or systematic lymph node sampling (eg, ≥3 N2 and ≥1 N1 stations).[3-7]**
- **In patients who do not smoke, small lesions that are presumed to be small cell carcinoma on biopsy should be resected because they are likely carcinoids that have been misdiagnosed (see NCCN Guidelines for Neuroendocrine and Adrenal Tumors).**
- **Surgery may be considered for selected patients with T3 (based on size), N0 SCLC, if invasive mediastinal lymph node staging is negative.**
- **Intraoperative diagnosis of likely SCLC in a patient with no prior biopsy**
  - **Mediastinal lymph node dissection or systematic lymph node sampling with frozen section is recommended to assess extent of disease and overall burden of disease.**
  - **If primary site and lymph nodes appear resectable, perform anatomic resection, preferably lobectomy. Pneumonectomy should not be performed if needed to encompass nodal metastatic disease.**
- **Patients who undergo complete resection should be treated with postoperative systemic therapy.[8] Patients without nodal metastases should be treated with systemic therapy alone. Patients with N2 or N3 nodal metastases should be treated with postoperative concurrent or sequential systemic therapy and mediastinal RT. Patients with N1 nodal metastases may be considered for postoperative mediastinal radiation.**
- **The benefit of PCI is unclear in patients who have undergone definitive therapy for pathologic stage I (T1–2a,N0,M0); see SCL-F.**

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PRINCIPLES OF SUPPORTIVE CARE

- **Smoking cessation advice, counseling, and pharmacotherapy**
  - **Use the 5 A's Framework: Ask, Advise, Assess, Assist, Arrange (https://www.ahrq.gov/prevention/guidelines/tobacco/5steps.html)**
  - **See NCCN Guidelines for Smoking Cessation**

- **Granulocyte-macrophage colony-stimulating factor (GM-CSF) or granulocyte colony-stimulating factor (G-CSF) are not recommended during concurrent systemic therapy plus RT (category 1 for not using GM-CSF).[1,2]**
- **Trilaciclib or G-CSF may be used as prophylactic options to decrease the incidence of chemotherapy-induced myelosuppression when administering platinum/etoposide ± immune checkpoint inhibitor (ICI)-containing regimens or a topotecan-containing regimen for extensive-stage SCLC (ES-SCLC).**

- **SIADH**
  - **Fluid restriction**
  - **Saline infusion for symptomatic patients**
  - **Demeclocycline**
  - **Vasopressin receptor inhibitors (ie, conivaptan, tolvaptan) for refractory hyponatremia**

- **Cushing syndrome**
  - **Consider ketoconazole. If not effective, consider metyrapone.**
  - **Consider referral to an appropriate endocrinology subspecialist.**

- **Leptomeningeal disease: See NCCN Guidelines for Central Nervous System Cancers**

- **Lambert-Eaton myasthenic syndrome**
  - **Consider amifampridine or intravenous immunoglobulin (IVIG) in consultation with neurology**

- **Pain management: See NCCN Guidelines for Adult Cancer Pain**

- **Nausea/vomiting: See NCCN Guidelines for Antiemesis**

- **Psychosocial distress: See NCCN Guidelines for Distress Management**
  - **Refer to the NCCN Distress Thermometer and Problem List, which includes social determinants of health**

- **See NCCN Guidelines for Palliative Care as indicated**

- **Consider early subspecialty consultation for unusual paraneoplastic neurologic syndromes to ensure the most recent management is done**

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PRINCIPLES OF SYSTEMIC THERAPY

PRIMARY OR ADJUVANT THERAPY FOR LIMITED-STAGE SCLC:

Four cycles of cytotoxic chemotherapy are recommended.
Planned cycle length should be every 21–28 days during concurrent RT.
The use of myeloid growth factors is not recommended during concurrent cytotoxic chemotherapy plus RT (category 1 for not using GM-CSF).[1]

Preferred

- Cisplatin 75 mg/m² Day 1 and Etoposide 100 mg/m² Days 1, 2, 3[2]
- Cisplatin 60 mg/m² Day 1 and Etoposide 120 mg/m² Days 1, 2, 3[3]
- Carboplatin area under the curve (AUC) 5–6 Day 1 and Etoposide 100 mg/m² Days 1, 2, 3[a,4]

- Consolidation Therapy
  - Durvalumab 1500 mg Day 1 every 28 Days for up to 24 months (category 1)[b,5]

Other Recommended

- Cisplatin 25 mg/m² Days 1, 2, 3 and Etoposide 100 mg/m² Days 1, 2, 3[2]

PRIMARY THERAPY FOR EXTENSIVE-STAGE SCLC[c]:

Four cycles of cytotoxic chemotherapy are recommended, but some patients may receive up to 6 cycles based on response and tolerability after 4 cycles.

Preferred

- Carboplatin AUC 5 Day 1 and Etoposide 100 mg/m² Days 1, 2, 3 and Atezolizumab 1200 mg Day 1 every 21 days x 4 cycles followed by maintenance Atezolizumab 1200 mg Day 1, every 21 days (category 1 for all)[d,e,f,6]
- Carboplatin AUC 5 Day 1 and Etoposide 100 mg/m² Days 1, 2, 3 and Atezolizumab 1200 mg Day 1 every 21 days x 4 cycles followed by maintenance Atezolizumab 1680 mg Day 1, every 28 days[d,e,f]
- Carboplatin AUC 5 Day 1 and Etoposide 100 mg/m² Days 1, 2, 3 and Atezolizumab 1200 mg Day 1 every 21 days x 4 cycles followed by maintenance Lurbinectedin 3.2 mg/m² and Atezolizumab 1200 mg Day 1, every 21 days (category 1 for all)[g,h,7]
- Carboplatin AUC 5–6 Day 1 and Etoposide 80–100 mg/m² Days 1, 2, 3 and Durvalumab 1500 mg Day 1 every 21 days x 4 cycles followed by maintenance Durvalumab 1500 mg Day 1 every 28 days (category 1 for all)[d,e,i,8]
- Cisplatin 75–80 mg/m² Day 1 and Etoposide 80–100 mg/m² Days 1, 2, 3 and Durvalumab 1500 mg Day 1 every 21 days x 4 cycles followed by maintenance Durvalumab 1500 mg Day 1 every 28 days (category 1 for all)[d,e,i,8]

Other Recommended

- Carboplatin AUC 5–6 Day 1 and Etoposide 100 mg/m² Days 1, 2, 3[9]
- Cisplatin 75 mg/m² Day 1 and Etoposide 100 mg/m² Days 1, 2, 3[10]
- Cisplatin 80 mg/m² Day 1 and Etoposide 80 mg/m² Days 1, 2, 3[11]
- Cisplatin 25 mg/m² Days 1, 2, 3 and Etoposide 100 mg/m² Days 1, 2, 3[12]

Useful in Certain Circumstances

- Carboplatin AUC 5 Day 1 and Irinotecan 50 mg/m² Days 1, 8, 15[13]
- Cisplatin 60 mg/m² Day 1 and Irinotecan 60 mg/m² Days 1, 8, 15[14]
- Cisplatin 30 mg/m² Days 1, 8 and Irinotecan 65 mg/m² Days 1, 8[15]

Footnotes (SCL-E 2 of 6)
Subsequent Systemic Therapy (SCL-E 3 of 6)
References (SCL-E 5 of 6)

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PRINCIPLES OF SYSTEMIC THERAPY – FOOTNOTES

[a] Cisplatin contraindicated or not tolerated.
[b] Those who did not experience disease progression after systemic therapy + concurrent RT may continue durvalumab until disease progression or intolerable toxicity, or for a maximum of 24 months.
[c] For transformation to SCLC from NSCLC, consider referral to a center with expertise (SCL-E 4 of 6).
[d] Contraindications for treatment with PD-1/PD-L1 inhibitors may include active or previously documented autoimmune disease and/or concurrent use of immunosuppressive agents. If TKI is continued for transformed SCLC from NSCLC with oncogenic driver, ICI should be avoided due to known toxicity.
[e] Maintenance immunotherapy with either atezolizumab or durvalumab should continue until progression or intolerable toxicity.
[f] Atezolizumab and hyaluronidase-tqjs subcutaneous injection may be substituted for IV atezolizumab. Atezolizumab and hyaluronidase-tqjs has different dosing and administration instructions compared to atezolizumab for intravenous infusion.
[g] Consider adding lurbinectedin to maintenance atezolizumab in patients who have achieved at least stable disease following four cycles of induction chemoimmunotherapy, have an ECOG PS of 0–1, and no history of brain metastases.
[h] This regimen is not for use as re-treatment in subsequent-line therapy if lurbinectedin has been used previously.
[i] Included patients with asymptomatic untreated brain metastases.

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SCLC SUBSEQUENT SYSTEMIC THERAPY (PS 0–2)[j] Consider dose reduction or growth factor support for patients with PS 2
Preferred
• Tarlatamab-dlle[k,l,16] (category 1)
• Clinical trial enrollment
• Irinotecan[m,17,18]
• Lurbinectedin (if not previously used)[19,20]
• If prolonged disease-free time, re-treatment with platinum-based doublet with or without immunotherapy[21-25]
• Topotecan Oral (PO) or Intravenous (IV)[17,23,26-28]
Other Recommended
• CAV (Cyclophosphamide/Doxorubicin/Vincristine)[26]
• Docetaxel[29]
• Gemcitabine[30,31,32]
• Nivolumab[n] or Pembrolizumab[o] (if not previously treated with an ICI)[d,33-36]
• Oral Etoposide[37,38]
• Paclitaxel[39,40]
• Temozolomide[40-42]

[d] Contraindications for treatment with PD-1/PD-L1 inhibitors may include active or previously documented autoimmune disease and/or concurrent use of immunosuppressive agents. If TKI is continued for transformed SCLC from NSCLC with oncogenic driver, ICI should be avoided due to known toxicity.
[j] Subsequent systemic therapy refers to second-line and beyond therapy.
[k] For extensive stage with disease progression on or after platinum-based chemotherapy.
[l] NCCN Guidelines for Management of CAR T-Cell and Lymphocyte Engager-Related Toxicities.
[m] For patients with CNS disease, consider using irinotecan.
[n] Nivolumab and hyaluronidase-nvhy subcutaneous injection may be substituted for IV nivolumab. Nivolumab and hyaluronidase-nvhy has different dosing and administration instructions compared to IV nivolumab.
[o] Pembrolizumab and berahyaluronidase alfa-pmph subcutaneous injection may be substituted for IV pembrolizumab. Pembrolizumab and berahyaluronidase alfa-pmph has different dosing and administration instructions compared to IV pembrolizumab.

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References on (SCL-E 5 of 6)

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PRINCIPLES OF SYSTEMIC THERAPY

- **Transformed SCLC from NSCLC with an Oncogenic Driver**
  - **This is a rare population of patients with very limited data to guide treatment.[43-46]**
  - **Systemic cytotoxic chemotherapy is recommended using the NCCN Guidelines for Small Cell Lung Cancer.[43,44]**
  - **The role of immunotherapy in this setting is unclear based on limited data.[43-46]**
  - **If tyrosine kinase inhibitor (TKI) is continued, ICI should be avoided, due to known toxicity.[45,47,48]**
  - **Consider referral to a center with experience managing transformed SCLC.**

References on (SCL-E 5 of 6)

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